

Fall Prevention Program

Risk Assessment, Prevention Interventions, and Post-Fall Management

Regulation: 42 CFR §482.13(e) | CMS Quality Measure

POLICY OVERVIEW

Patient falls are a leading cause of injury in hospitals and a key patient safety metric. An effective fall prevention program requires systematic risk stratification, individualized prevention interventions, a just culture of reporting, and structured post-fall analysis to drive continuous improvement.

KEY REQUIREMENTS

1 Fall Risk Assessment

A validated fall risk assessment tool (Morse Fall Scale, STRATIFY, or equivalent) must be completed on admission, after any fall, after a change in condition, and with each shift assessment. Risk level must be documented and communicated during handoffs.

2 Universal Fall Precautions

All patients, regardless of risk score, must receive: bed in lowest position, call light within reach, non-slip footwear, clear path to bathroom, and orientation to environment. Environmental safety rounds must be conducted each shift.

3 High-Risk Interventions

High fall-risk patients require additional interventions including: yellow fall-risk armband, bed/chair alarm, hourly rounding, toileting schedule, fall-risk signage, and removal of unnecessary equipment from room. Consider bed exit sensor alarms for highest-risk patients.

4 Post-Fall Management

Following any fall: ensure patient safety and assess for injury, notify physician, complete incident report, perform post-fall huddle within 24 hours with bedside team, and conduct root cause review. CT head/imaging as ordered for head strikes.

5 Patient and Family Education

Patients and families must be educated on fall risk, the importance of calling for assistance, how to use the call light, and safe mobility techniques. Education must be documented and reinforced throughout hospitalization.

COMPLIANCE NOTES

Falls with injury are a CMS Hospital-Acquired Condition (HAC) that can result in reduced Medicare reimbursement. Fall rate and fall with injury rate should be tracked, benchmarked, and reported to quality committees and the Governing Board.

COMMON SURVEY CITATIONS

Cited under §482.13(e) – Safe environment standards, and tracked as a CMS Patient Safety Indicator (PSI-08).